

SDOH and HRSN Reference Guide

Healthy People 2030 • CDC • CMS • PRAPARE • ADA • AHA/ACC
Prepared from official and professional source pages | Retrieved August 12, 2026

Purpose and scope

This guide consolidates a practical framework for healthcare and payer analytics: community-level social determinants of health (SDOH), individual-level health-related social needs (HRSN), screening, referral workflows, and chronic-disease guidance. It is a research and implementation reference, not a substitute for current clinical judgment, payer policy, or the full source documents.

Clinical guidance changes over time. Verify the latest ADA Standards of Care and AHA/ACC guideline before using thresholds or recommendations in patient care, quality measurement, or production decision support.

1. Healthy People 2030: SDOH framework

Healthy People 2030 defines SDOH as the conditions in the environments where people are born, live, learn, work, play, worship, and age that affect health, functioning, quality of life, and risks. Its five domains are:

- Economic stability — employment, income, food security, housing stability and related financial conditions.
- Education access and quality — early childhood education, school quality, literacy and educational opportunity.
- Health care access and quality — coverage, access to care, care quality and health information.
- Neighborhood and built environment — housing, transportation, safety, environmental conditions and physical activity opportunities.
- Social and community context — social cohesion, discrimination, civic participation, violence and relationships.

Healthy People 2030 emphasizes upstream conditions and includes the overarching goal of creating social, physical, and economic environments that promote the full potential for health and well-being for all. Examples include safe housing and transportation, nutritious food access, clean air and water, education, income, language and literacy, discrimination and violence.

2. CDC: public-health perspective

CDC describes SDOH as nonmedical factors influencing health outcomes, shaped by broad forces and systems such as economic policies, social norms, social policies and political systems. CDC connects SDOH work to health equity: differences in social conditions can create avoidable and unjust differences in health.

Public-health actions that affect SDOH include:

- Convene communities and organizations to identify local concerns and assets.
- Integrate multiple data sources, including public-health, geographic and environmental data, to guide strategy.
- Influence policy and funding decisions that address priority conditions.
- Collaborate across sectors to implement and scale solutions.

For analytics, this supports combining clinical and claims data with community indicators, stratifying outcomes by geography and population characteristics, and evaluating whether interventions improve access and equity.

3. CMS: payer and HRSN context

CMS distinguishes community-level SDOH from individual-level HRSN. HRSN are social and economic needs experienced by individuals that can affect health and health-care use. Common examples include food, housing and

utilities, transportation, financial strain, safety, and access to care.

A useful analytics distinction is: SDOH describes the surrounding conditions; HRSN describes a person's identified need. They can overlap—for example, area-level economic disadvantage may coexist with an individual's inability to pay for medication.

CMS Innovation Center models may screen for HRSN, connect people with community resources, use care navigators or community health workers, and focus on needs in specific populations or care settings. For payer analytics, useful measures include screening completion, positive-need rates, referral acceptance, service connection, closed-loop referral, medication access, appointment completion, utilization and outcomes.

4. PRAPARE: SDOH screening

PRAPARE (Protocol for Responding to and Assessing Patients' Assets, Risks and Experiences) is a national standardized patient risk-assessment tool designed to engage patients in assessing and addressing SDOH. NACHC describes it as evidence-based, developed through stakeholder engagement, paired with an Implementation and Action Toolkit, and standardized across ICD-10, LOINC and SNOMED. It is available in more than 25 languages.

Implementation considerations:

- Use screening as a conversation, not merely a data-collection event.
- Capture patient assets as well as risks and experiences.
- Record consent, preferred language, accessibility needs and the date of assessment.
- Map positive findings to appropriate local resources and document referral status.
- Monitor completion and resolution separately; a referral is not the same as a successfully addressed need.
- Apply privacy, minimum-necessary access and role-based governance to sensitive social information.

5. ADA: diabetes guidance context

The ADA Standards of Care in Diabetes is updated annually and provides current clinical recommendations, treatment goals and tools for evaluating quality of care. Relevant sections include population health, prevention, comprehensive assessment, health behaviors and well-being, glycemic goals, cardiovascular risk, kidney disease, older adults and advocacy.

For an SDOH/HRSN project, diabetes workflows should consider food security and nutrition access, medication affordability, transportation, housing stability, health literacy, behavioral health, social support and technology access. These factors can affect screening, self-management, medication adherence, glucose monitoring and follow-up.

Operationally, link SDOH findings to diabetes measures only with appropriate clinical and analytic safeguards. Avoid treating a social need as patient nonadherence without accounting for affordability, access and care-plan barriers. Use shared decision-making and individualized goals from the current ADA Standards.

6. AHA/ACC: hypertension and BP guidance

The 2025 AHA/ACC adult high-blood-pressure guideline replaces the 2017 guideline. The published BP categories remain: normal below 120/80 mm Hg; elevated systolic 120–129 with diastolic below 80; stage 1 hypertension 130–139 or 80–89; and stage 2 hypertension at least 140 or at least 90 mm Hg.

The guideline emphasizes prevention, healthy lifestyle behaviors, home BP monitoring, risk-based treatment, and reducing cardiovascular, kidney, cognitive and other complications. It also introduces use of the AHA PREVENT risk calculator and highlights the relevance of social drivers, including ZIP code as a proxy in risk estimation.

For analytics, distinguish office readings, home readings and confirmed diagnoses; retain both systolic and diastolic values; track repeat measurement and treatment intensification; and stratify control by access, medication affordability,

transportation, food environment and other relevant HRSN. Do not use a single BP value as a diagnosis.

7. Crosswalk for a healthcare analytics project

Layer	Primary question	Example data elements
Community SDOH	What conditions shape health in the patient's environment?	ZIP/census geography, food access, housing, transportation, education, pollution, neighborhood safety
Individual HRSN	What social need is affecting this person now?	Screening responses, financial strain, food/housing/utility/transportation need, safety, language
Clinical risk	What health condition or risk is present?	A1C/glucose, BP, medications, kidney markers, comorbidities, utilization
Action and outcome	Was the need addressed and did care improve?	Referral, acceptance, service connection, closed-loop status, adherence, BP/A1C control, ED use

Source list

Healthy People 2030 — Social Determinants of Health

<https://health.gov/healthypeople/priority-areas/social-determinants-health>

CDC — Public Health Actions That Address SDOH

<https://www.cdc.gov/about/priorities/why-is-addressing-sdoh-important.html>

CMS — Social Drivers of Health and HRSN

<https://www.cms.gov/priorities-innovation-key-concepts-social-drivers-health-health-related-social-needs>

NACHC — PRAPARE

<https://www.nachc.org/resource/prapare/>

American Diabetes Association — Standards of Care in Diabetes

<https://professional.diabetes.org/standards-of-care>

AHA/ACC — 2025 High Blood Pressure Guideline announcement

<https://www.acc.org/about-acc/press-releases/2025/08/13/20/03/new-high-blood-pressure-guideline-emphasizes-prevention-early-treatment-to-reduce-cvd-risk>

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